PARTAMOL TAB. TABLETS 500MG

1. Name of the medicinal product
PARTAMOL TAB. TABLETS 500MG

2. Special notice and recommendation
Keep out of reach of children
Read the package insert carefully before use

3. Composition
Each tablet contains:
Paracetamol .... 500 mg

4. Pharmaceutical form
Tablet.
White, round-shaped tablet, engraved “PARACETAMOL” on one side and breakable line on the other side.

5. Indications
Paracetamol is a mild analgesic and antipyretic, and is recommended for the treatment of most painful and febrile conditions, for example, headache including migraine and tension headaches, toothache, backache, rheumatic and muscle pains, dysmenorrhoea, sore throat, and for relieving the fever, aches and pains of colds and flu. Also recommended for the symptomatic relief of pain due to non-serious arthritis.

6. Administration and dosage
Route of administration
Oral administration only.
Dosage
Do not exceed the stated dose.
The lowest dose necessary to achieve efficacy should be used for the shortest duration of treatment.
Minimum dosing interval: 4 hours.
Maximum daily dose: 4000mg (8 tablets).
Adults (including the elderly) and children aged 12 years and over
- 1 to 2 tablets (500 mg to 1000 mg paracetamol), taken every 4 to 6 hours as required.

Children 6 to 11 years
- No more than 4 doses in any 24-hour period.
- Maximum duration of continued use without medical advice: 3 days.
- Maximum daily dose: 60mg/kg to be administered in divided doses of 10-15 mg/kg throughout the 24-hour period.
- *Children 6 to 8 years:* ½ tablet (250 mg)
- *Children 9 to 11 years:* 1 tablet (500 mg)

Children under 6 years:
- Not recommended for children under the age of 6 years.

Renal impairment and hepatic impairment
- Patients who have been diagnosed with renal impairment or liver impairment must seek medical advice before taking this medication. The restrictions related to the use of paracetamol products in patients with renal impairment or liver impairment are primarily a consequence of the paracetamol content of the drug.

7. Contraindications
Hypersensitivity to paracetamol or any of the other constituents.

8. Special warnings and precautions for use
- Underlying liver disease increases the risk or paracetamol related liver damage. Patients who have been diagnosed with liver or kidney impairment must seek medical advice before taking this medication.
- Do not exceed the stated dose.
- Patients should be advised to consult their doctor if their headaches become persistent.
- Patients should be advised to consult a doctor if they suffer from non-serious arthritis and need to take painkillers every day.
- Caution should be exercised in patients with glutathione depleted states, as the use of paracetamol may increase the risk of metabolic acidosis.
- Use with caution in patients with glutathione depletion due to metabolic deficiencies.
- If symptoms persist, medical advice must be sought.
- Keep out of the sight and reach of children.

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time.

ALLERGY ALERT: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters and rash.
These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

9. Pregnancy and lactation
Epidemiological studies on neurodevelopment in children exposed to paracetamol in utero show inconclusive results. If clinically needed, paracetamol can be used during pregnancy, however, as with any medicine it should be used at the lowest effective dose for the shortest possible time.
Paracetamol is excreted in breast milk but not in a clinically significant amount in recommended dosages. Available published data do not contraindicate breastfeeding.

10. Effects on ability to drive and use machines
None.

11. Drug interactions
The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by colestyramine. The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

12. Adverse reactions
Adverse events of paracetamol from historical clinical trial data are both infrequent and from small patient exposure.
Accordingly, events reported from extensive post-marketing experience at therapeutic/labelled dose and considered attributable are listed below by system class and frequency.
The following convention has been utilised for the classification of the undesirable effects: very common (≥1/10), common (≥1/100 to <1/10), uncommon (≥1/1000 to <1/100), rare (≥1/10,000 to <1/1000) and very rare (<1/10,000), not known (cannot be estimated from available data).
Adverse event frequencies have been estimated from spontaneous reports received through post-marketing data.
Blood and lymphatic system disorders
Very rare: Thrombocytopenia; Agranulocytosis.
Immune system disorders
Very rare: Anaphylaxis.
Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.
Respiratory, thoracic and mediastinal disorders
Very rare: Bronchospasm *.
Hepatobiliary disorders
Very rare: Hepatic dysfunction.
* There have been cases of bronchospasm with paracetamol, but these are more likely in asthmatics sensitive to aspirin or other NSAIDs.

13. Overdosage and management
Paracetamol overdose may cause liver failure which may require liver transplant or lead to death.
Liver damage is possible in adults who have taken 10g or more of paracetamol. Ingestion of 5g or more of paracetamol may lead to liver damage if the patient has risk factors.
Risk factors
If the patient:
- Is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes, or
- Regularly consumes ethanol in excess of recommended amounts, or
- Is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms
Symptoms of paracetamol overdosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.
Management
Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section.
Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24h from ingestion should be discussed with the NPIS or a liver unit.

14. Pharmacodynamic properties
Pharmacotherapeutic group: Other analgesics and antipyretics; Anilides
ATC code: N02BE01
Paracetamol is an antipyretic analgesic. The mechanism of action is probably similar to that of aspirin and dependant on the inhibition of prostaglandin synthesis. This inhibition appears, however to be on a selective basis.

15. Pharmacokinetic properties
Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract. The concentration in plasma reaches a peak in 30 to 60 minutes and the plasma half-life is 1 - 4 hours after therapeutic doses. Paracetamol is relatively uniformly distributed throughout most body fluids. Binding of the drug to plasma proteins is variable; 20 to 30% may be bound at the concentrations encountered during acute intoxication. Following therapeutic doses 90 - 100% of the drug may be recovered in the urine within the first day. However, practically no paracetamol is excreted unchanged and the bulk is excreted after hepatic conjugation.

16. Packaging
Blister of 10 tablets. Box of 10 blisters.

17. Storage condition, shelf-life
Store in a well-closed container, in a dry place, protect from light. Do not store above 30°C.
The expiry date of this pack is printed on the box. Do not use this pack after this date.

18. Name, address of manufacturer

STELLA

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